CISMUN 2025 Committees

- DISEC
- UNWOMEN
- WHO
- UNEP
- UNESCO
- UNHRC
- ECOSOC
- FIFA
- UNSC
- UNSC (Crisis)
- USCC
- Village Council

MUN Procedures

- Committee Session
    1) Roll Call
        - All countries called, delegate responds with “Present” or “Present and Voting”
    2) General Speaker’s List
        - Chosen delegates give opening speech
    3) Moderated Caucus
        - Speaking rules apply
    4) Unmoderated Caucus
        - Informal session, delegates can walk around and talk to other delegates
    5) Drafted Resolutions
        - Done with multiple delegates
    6) Voting
        - Delegates vote for best resolution

MUN Terminology

- Moderated Caucus
    - Short statements/comments from delegates on sub-issues
    - If someone wants to speak on the subject, they must raise their placard and wait for the chair to call on them
- Unmoderated Caucus
    - Free movement
    - Talk to other delegates
    - Form alliances
    - Brainstorm resolutions

- Floor
    - Opportunity to speak
- Motion
    - Suggest an action
        - Extend debate time
        - Vote on resolutions
- Second
    - Agree with another delegate's motion
- Objection
    - Disagree with another delegate's motion
- Abstain
    - If a delegate is unsure about their vote they can choose not to say yes or no definitively
        - Does not fully agree
        - Agrees enough not to vote against

MUN Points

- Question or suggestion made by a delegate to the chair
- Usually regarding their comfort
- Types of points :
    - Point of Order
        - Objection to a procedure
        - The chair has made a mistake
        - Parliamentary Procedures are not done correctly
        - Must be made
        - Must say point of order whilst raising their placard
        - May interrupt a speaker who has the floor

- Point of Inquiry
    - For clarification
- Point of Information
    - To add onto another speaker statement/resolution
    - To bring down the resolution
    - Must be formulated as a short question
    - May NOT interrupt speaker who has the floor
    - Must be directed to the chair, even though it is aimed at the speaker
    - Max 2 times
- Point of Personal Privilege
    - When a delegate feels uncomfortable
    - Room too hot / too cold
    - May interrupt speaker to request for change in atmosphere
- Point of Parliament Inquiry
    - Point of info directed at chair regarding the rules of procedure

Palliative Care Stances

Country	Bloc	Unique Stance

Argentina	***Bloc B*** ***(UMIC)***	**National Palliative Care Law** (Law No. 27.678)
		- Aims for **comprehensive palliative care** across **healthcare sectors**
		- Aims to **provide support** for **families**
		Patients' Rights Law (Law No. 26.742)
		- Allows patients to :
		  - **Refuse** surgery
		  - **Refuse** reanimation
		  - **Refuse** hydration or nutrition
		Promotes access to **essential pharmacological therapies** (opioids)
		
		Lacks : a national program, initiatives to improve rural access, methods of rectifying the shortage in training, education, and professionals

Australia	***Bloc A*** ***(HIC)***	**"People affected by life-limiting illnesses get the care they need to live well"** **Person-centered** approach Can be provided at **any age**, at **any stage** (of illness), and **alongside curative treatments** Belief that **everyone has a role to play** (healthcare professionals of various specialties, communities, and individuals) Care is **high-quality** and **evidence based** **Goals include :** - Further **integration** into primary care - Greater **investment** in home/community care **Aged care priority** – considered "core business" for residential and in-home caregivers **National Palliative Care Standards** – created by the **PCA** (Palliative Care Australia) - Guide for **best practice**s Development and **improvement** of **quality care** - **Self assessments** - Focus on **appropriate care for diverse populations** **Strong**, **coordinated** national policy **Early integration** of palliative care is being **actively developed** **Aims** to be **accessible** in **communities** and **homes** Lacking : recognition of opioid liberalization
Belgium	***Bloc A*** ***(HIC)***	
Canada	***Bloc A*** ***(HIC)***	**Improving access, quality, and consistency** "All Canadians with serious illness live well until the end of life" **4 Focus Areas** - **Training** and **education** for **healthcare professionals** - **Support** for healthcare providers

Country	Bloc	Unique Stance
		- **Research** and **data collection** of care - **Facilitating equitable access** across Canada **Challenges include :** - **Rural/remote** access - **Indigenous** communities - **Non-cancer** diagnoses - Insufficient number of **patient beds** Required **before** an application for **MAID** is allowed
China	***Bloc B*** ***(UMIC)***	**Supports palliative care programs** - Recognition of palliative care importance was (relatively) late **14th 5-year plan** to **improve** elderly services with emphasis on **end-of-life care** **NHC** () issued **guidelines** on : **structure, human resources, environment, quality monitoring,** and **practice** **Encourages access** to **essential medicines** (**opioids**) by including **medication coverage**, as well as **issuing regulations** on **dangerous substances** **Cultural influence** may cause people to feel they are **giving up** and may feel a strong **opposition** to **death** – leads to **refusal** of **palliative care** (when it is the most appropriate) **Aims** to **improve quality** of **end-of-life care** through **pain** and **symptom management**, **comfort care, psychological, spiritual,** and **social support** **Lacks** : funding for palliative care, professionals and training programs, access to facilities
Colombia	***Bloc A*** ***(UMIC)***	**Progressive legal framework** - Palliative care is seen as a **fundamental patient right** **Challenges include :** - **Equitable implementation** and **access** (especially in **rural** areas) - **Lack** of **trained palliative care professionals** - Attempts to right this have been made with a **2022 bill** to enhance **nationwide palliative care education** Palliative care is described in **legislation** – Law 1733 of 2014 (Ley Consuelo Davis Saavedre) - Defines palliative care as; improving quality of life through **physical**, **psychosocial**, **emotional**, **social**, and **spiritual** factors Palliative care "**affirms life and considers dying a normal process**" **EPS** and **IPS** are required to **guarantee access** to **palliative care services** Law specifically mentions the **guarantee of 24/7 distribution of opioid medications** for **pain management** – addressing a **common barrier** in many low to middle income countries Comprehensive **5-year public health action plan** to improve **equitable access** to palliative care across the nation Currently care is still carried out **partially** by **specialized palliative care professionals** – **preventing** the **integration** of care and the **implementation** of community based care
France		

Country	Bloc	Unique Stance
Germany		
Ghana		
India		
Italy		
Japan		
Kenya		
Netherlands		
Norway		
Poland		
Portugal		
Russia		
South Africa		
Spain		
Sweden		
Thailand		
Uganda		
United Kingdom		

Country	Bloc	Unique Stance
United States	***Bloc B*** ***(HIC)***	**Hospice care** is NOT palliative care – involves **curative treatments** **Palliative care** may occur at the **same time as curative care** **Palliative care** is commonly accessed through **fee-for-service** models **Care programs** for **palliative treatments** may exist in **hospitals** **PCHETA -** *Palliative Care and Hospice Education and Training Act* **Goals include** : - **Increasing public awareness** - **Mandatory training** for healthcare professionals - **Adopt policies** that allow palliative and curative care to occur simultaneously **The Clinical Practice Guidelines** exist for quality palliative care **Quality oversight** **Palliative care** occurs in the **last few months of life** Lacking : recognition of a need for service coverage

Country	Bloc	Unique Stance
Argentina	***Bloc B***	Active euthanasia is illegal. A “dignified death” framework recognizes the patient’s right to refuse or discontinue life-prolonging treatment and to receive palliative sedation. Advance directives are recognized; assisted suicide remains prohibited.
Australia	***Bloc A***	All states have Voluntary Assisted Dying (VAD) laws in force. Common features: adult, decision-making capacity, advanced disease causing intolerable suffering, and expected death within a set timeframe (often 6–12 months; longer for neurodegenerative diseases). Assessment by two independent clinicians; self-administration preferred with practitioner administration as backup.
Belgium	***Bloc A***	Voluntary euthanasia (doctor-administered) and assisted suicide are legal under a 2002 law for competent patients with constant, unbearable suffering (somatic or psychiatric) with no reasonable alternative. Multiple safeguards: repeated requests, second-doctor confirmation (third for non-terminal or psychiatric cases). Minors may qualify under very strict conditions with parental consent.
Canada	***Bloc A***	Nationwide MAID (Medical Assistance in Dying) allows both clinician-administered and self-administered options for adults who are capable, give informed consent, and have a grievous and irremediable condition. Terminal diagnosis is not always required; separate tracks exist for end-of-life vs. non-end-of-life cases, with extra safeguards (waiting periods, expert consults). Ongoing debates about eligibility for mental illness as a sole condition.
China	***Bloc B***	Euthanasia and assisted suicide are prohibited. End-of-life decision-making focuses on palliative care and family-doctor consensus; advance directives are not widely formalized in national law.

Colombia	***Bloc A***	Euthanasia has been permitted via Constitutional Court jurisprudence since the late 1990s, later regulated by the health ministry. Applies to competent adults with terminal illness and unbearable suffering; later rulings broadened beyond strictly terminal in some circumstances. Institutional committees review each case.
France	***Bloc B***	As of now, euthanasia and assisted suicide are not legal, but the Leonetti-Claeys framework allows deep, continuous sedation until death and withdrawal of treatment for patients with short-term prognosis and refractory suffering. A government “aid in dying” bill is under active debate to create a narrow assisted-dying pathway with safeguards.
Germany	***Bloc B***	Active euthanasia (killing on request) remains criminal. The Constitutional Court (2020) struck down a ban on “commercial” assisted suicide support, recognizing a right to a self-determined death; Parliament continues to debate regulatory frameworks. In practice, assisted suicide is possible in narrow, carefully supervised circumstances; palliative sedation and withdrawal of treatment are legal.
Ghana	***Bloc B***	
India	***Bloc B***	The Supreme Court recognized passive euthanasia (withdrawal of life support for patients in persistent vegetative state/terminal illness) with advance directives permitted under detailed procedural safeguards. Active euthanasia and assisted suicide remain illegal.
Italy	***Bloc B***	Active euthanasia is illegal. Following a Constitutional Court decision, assisted suicide may be non-punishable in very restricted cases (e.g., patient is kept alive by life-support, is fully capable, and suffers intolerably), subject to ethics-committee review. Attempts to broaden access via referendum or statute have not succeeded.

Japan	***Bloc B***	Comprehensive statute. Courts have set limited criteria under which passive euthanasia or termination of futile treatment is permissible with patient consent/advance directive. Active euthanasia and physician-assisted suicide are generally criminal; rare, controversial cases have appeared.
Kenya	***Bloc B***	Euthanasia and assisted suicide are illegal. Medical ethics guidance supports palliative care and withholding/withdrawing futile interventions under best-interests standards; no statutory assisted-dying framework.
Netherlands	***Bloc A***	
Norway	***Bloc B***	Active euthanasia and assisted suicide are criminal offences, though penalties can be mitigated for killing on request. Withdrawal of futile treatment and palliative sedation are permitted within clinical ethics frameworks.
Poland	***Bloc B***	Euthanasia and assisted suicide are illegal and punishable; courts may reduce sentences for killing on request but the acts remain crimes. Strong emphasis on hospice and palliative care; advance directives have limited legal effect.
Portugal	***Bloc A***	Parliament approved a law decriminalising medically assisted death after multiple constitutional reviews; the final text limits eligibility to adults with severe, irreversible suffering, typically prioritising assisted suicide with euthanasia as a fallback. Implementation requires strict procedures and oversight commissions.
Russia	***Bloc B***	Euthanasia (including at a patient's request) is expressly banned by law; doctors must not intentionally shorten life. Withholding disproportionate treatment may occur case-by-case, but no legal assisted dying pathway exists.

South Africa	***Bloc B***	No statute permits euthanasia or assisted suicide. Courts have been approached in individual cases but there's no settled right; withdrawing or withholding futile treatment and robust palliative care are lawful. Law reform is periodically proposed but not enacted.
Spain	***Bloc A***	Since 2021, both euthanasia and assisted suicide are legal for adults with serious, chronic conditions causing unbearable suffering with no reasonable treatment alternatives. Requires repeated requests, independent medical opinions, and oversight by regional commissions.
Sweden	***Bloc B***	Active euthanasia is unlawful. Passive euthanasia—withdrawing treatment at a competent patient's request—is allowed; strong emphasis on patient autonomy, advance directives, and palliative care.
Switzerland	***Bloc A***	
Thailand	***Bloc B***	No legal route for euthanasia or assisted suicide. The National Health Act permits advance directives refusing life-sustaining treatment and supports palliative care; clinicians may withdraw futile treatment with proper documentation and family discussion.
Uganda	***Bloc B***	Euthanasia and assisted suicide are illegal. End-of-life care policy centres on palliative care access; clinicians may limit futile treatment but cannot intentionally hasten death.

United Kingdom	***Bloc B***	Euthanasia and assisted suicide are illegal (Suicide Act 1961 in England/Wales; similar in NI; Scotland uses common law). Prosecutorial guidelines allow public-interest discretion in some compassion cases, but there is no legal route; multiple bills have been debated without passage. Withdrawal of treatment and palliative sedation are lawful with consent/best-interests tests.
United States	***Bloc A***	Active euthanasia is illegal everywhere. Physician-assisted dying is legal by statute in a number of jurisdictions (e.g., OR, WA, CA, CO, HI, ME, NJ, NM, VT, DC, MT via court ruling); typically limited to terminal illness with ≤6 months' prognosis, two oral requests separated by waiting period, written request, and capacity checks. Interstate travel for access and telemedicine rules vary.

Palliative Care

- When **recovery** is no longer possible, the focus of care will be on **ensuring** that **quality of life** is at its maximum capacity
    - All patients with **terminal conditions** can receive palliative care
- **1997** palliative care networks developed
    - Palliative **home care teams**
    - Palliative **day care centers**
- **Hospitals** can deliver palliative care in **dedicated units** or **other departments** where the care plan is being made by a **mobile palliative team.**
- In Belgium, of all **terminal cancer** patients that died in 2020 the percentage of patients who received **palliative** care has risen to **57%**
- **⅕** cancer patients who received **palliative care died within a week**
    - Care was **implemented at a very late stage**
- Many **studies** show that most people would like to **die at home**
- Entitled to take **sabbatical leave** or **reduce** their **working hours** to provide **palliative care** to someone with an incurable disease
    - This person does not have to be a family member
- **Remote care** allow seniors and people suffering from illness to **regain and maintain independence**
- **Home care** can sometimes **prevent** the need for **hospital**
- **Belgium** states an **explicit right** to **palliative care** in their law
- **WHO** called for the **integration** of **palliative care** into **national health care systems**
    - Aims to let **patients** have **access** to **palliative care** without unnecessary **delays**
- **Mobile palliative care teams** may be tasked with **raising awareness** amongst **hospital staff**
    - All Belgian **general hospitals** and **other isolated departments** have a **mobile palliative care team**
    - Around half (50) **General Hospitals** have **palliative units** for **terminally ill individuals** that are near death
- **WHO states :**
    - **National health policies** and **systems** often do **not include palliative care** at all;
    - **Training** on palliative care for health professionals is often **limited or non-existent**;
    - Population access to **opioid pain relief** is **inadequate** and **fails** to **meet international conventions** on **access** to **essential medicines**.
- Belgium initiated (on behalf of the UN Commission on Narcotic Drugs)
    - **"No Patient Left Behind"** – an initiative that aims to **improve global access** to **medications**

- Belgium acknowledges the need for **liberalization** of **opioid medications** but the increasing number of **opioid abuse cases** has caused necessary **caution** when prescribing such addictive medications.

- Economic benefits stem from the following factors :
    - **Goals of care** are **discussed** and **discharging** of patients is **planned** which results in **shorter in-patient stays** for people with serious illness
    - **Patient preference** may **eliminate** certain high-cost **treatments** and **diagnostics** that have limited benefits at the end of life – listening to patient preference results in **fewer non-beneficial treatments**
    - **Repeat hospital admissions** may be **reduced** through **improved pain** and **symptom control** – as well as the access to **outpatient** and **home care services**
        - As **hospital admissions** account for a **large portion** of **health care costs** across one's lifetime
        - **Reductions** in **admissions** will **save** large amounts for the **system** and **increases availability** for other services and conditions

Curative Care

- Implies that **aggressive curative treatments** are **stopped** at the end of life (especially within the last **14 days**)
    - **Continued** aggressive curative treatments are an **inappropriate** manner of care in **terminally ill people.**
- **Need** for **acute care** beds
- **Shortages** occur when **supply** does **not** meet **demand**
    - **Critical impact** occurs when the **lack** of **medication** has a **severe impact** on **patients**
    - Medications targeting the **nervous system** and **cardiovascular system** had the **most critical impact notifications** (via the **FAGG/AFMPS**)
- During the **pandemic** there was a **significant decrease** in **diabetes** in **Belgium** and the **EU**

Euthanasia

- Belgium legalized **euthanasia** (**not assisted suicide**) in **September** of **2002**
    - For those experiencing **constant** and **unbearable physical** or **mental** suffering that **cannot be alleviated**
- **The Euthanasia Act**
    - Ensures that **physicians cannot be compelled to perform euthanasia** and **other persons cannot be forced to assist in it.**
    - If a physician **declines to perform the procedure the patient file must be transferred to a physician or person designated to the patient** (if asked to do so)
- **Change in attitudes** of doctors was **necessary** to ensure the **opportunity** of euthanasia is being **fairly offered**
- **Euthanasia is part of palliative care**
- Physicians are required to give **all information** that allows patients to **opt** for **euthanasia**
- Roughly **20-30 cases** per month after **initial implementation**
- "**Euthanasia** is one of the **possible choices** in **terminal care** and must be framed by and embedded in **total palliative care** that **transcends individual care**."
- "**Euthanasia** can be a **possible choice** in **terminal care**, framed by and embedded in a **palliative total care approach**."
- Avoiding **polarization** within the medical community → focus on **patient care** and what is **best for the patient**
- **Separating** the **procedure** from the **practice of medicine** altogether
- If **legalization** leads to the **redefinition** of **palliative care** possible conflicts may be greater
- Informed consent is a vital part of the euthanasia process, thus :
    - **objecting physicians** should give **timely notice** of their views to patients
    - **professional associations** should assist in **developing strategies** that will allow objecting physicians to deal with requests for euthanasia in a manner that is **respectful of the patient** and **consistent with their own principles**
    - **end-of-life discussions** should be **recorded** in the patient's medical file
    - an "**information and communication contract**" might be used to **document** the physician-patient understanding of end-of-life issues.
- An **objector** must be aware that he may **positively contribute** to a **choice** for **euthanasia** by **alienating** the patient.
- **Referrals** are **mandatory** for **euthanasia**
- Need to **balance autonomy** & **personal biases** (of physicians)
- Requirements to be euthanized
    - the euthanasia request is made by a **legally competent adult** patient

    - the request is **voluntary**, **repeated**, **well- considered**, and **not** the result of **external pressure**
    - the patient is in a **medical condition without prospect of improvement**
    - the patient experiences **constant** and **unbearable mental suffering** that cannot be **alleviated**
    - the **suffering** is the result of a **serious** and **incurable psychiatric** disorder
- Assessing this criteria requires the **consultation** of **2 independent physicians**, including a **psychiatrist**
- There must be a **waiting period** to ensure **certainty** of the **procedure**
- The **attending physician** must report the **procedure** to the **Federal Control and Evaluation Commission for Euthanasia**
- Main **diagnoses** for **euthanasia** include;
    - **Cancer** (54%)
    - **Polypathologies** (26.8%)
    - **Serious Neurological Disorders** (8.1%)
        - **Mood disorders** (55.7%)
        - **Personality disorders** (19.4%)
        - **Psychotic disorders** (6.2%)
        - Anxiety disorders and PTSD (6.2%)
        - Autism Spectrum Disorder (4.6%)
        - Eating disorders (1.5%)
        - Other combinations (6.5%)
- **Age demographics** of euthanized individuals
    - **72.6%** of patients **over 70**
    - **43.2%** of patients **over 80**
    - **1.3%** of patients **under 40**

- Euthanasia for mental suffering is legal in :
    - Belgium
    - The Netherlands
    - Spain
    - Colombia
    - Canada
    - Switzerland
    - (*Luxembourg*)

Legal Safeguards and Oversight (FCEC)

- All cases of euthanasia must be **reported** to the **FCEC** (Federal Control and Evaluation Commission for Euthanasia)
- **Authority** ensures all **proper procedures** were followed
- The **FCEC** also **collects data** and **publishes reports** on **euthanasia statistics** (while maintaining appropriate confidentiality)

Specific Conditions (of Euthanasia) for Terminal, Non-Terminal, and Minor Patients

- Main residence in Belgium
- **Age of legal majority (18)** or an **emancipated minor** (may apply through court from 15 years old)
- Capable of **expressing own will**
- Request (Advance Declaration) must :
    - Be made in **writing**
    - **Co-signed** (on the same day) by 2 adult witnesses, at least one of whom must have no material interest in their death
    - Be **voluntary**, **well-considered** and **repeated**, and is **not** the result of any **external pressure**
- Patient must be in a **medically futile** condition of **constant** and **unbearable physical** or **mental suffering** that **cannot** be **alleviated**
- Resulting from a **serious** and **incurable disorder** caused by **illness** or **accident**
- Patient may **appoint one** or **more trusted persons** (who must be adults)
- Service is **Free**
- **In certain circumstances :**
    - **Minors** may be **eligible** for **euthanasia** if**:**
    - They are **conscious,** have the **capacity** for **discernment** (ability to make wise, clear judgments and to perceive subtle differences between things)
    - Meet the general criteria of **constant** and **unbearable physical** or **mental suffering**
    - Have **written parental consent**
    - **Requires physician approval**

Key Terms

Palliative VS. Curative Care	
Term	***Definition***
Palliative Care	A **biopsychosocial** approach in healthcare, focusing specifically on improving a patient's **quality of life** and **well-being**; Examples of palliative care could be **managing pain** and **other symptoms**, while considering **psychological**, **spiritual**, and **social support** for the patient's environment.
Palliative Sedation	A method of **relieving refractory** and **intolerable suffering** (symptoms that cannot be controlled by any other means) by **inducing** a state of **decreased awareness**.
Curative Treatment	A **medical treatment** meant for **curing** an illness or disease with the **goal** of a patient's **full recovery**, often through **surgery**, **assigned medication**, or **specialized treatments** like **therapy**.
Hospice Care	A type of **palliative care**, specializing in providing care to patients, those in their **final stages of terminal illnesses**. It focuses more on **enhancing comfort and dignity** rather than using **aggressive treatments.**
End-Of-Life Care	**Healthcare services** provided during the period when a patient is **nearing death**, which may include both **palliative** and **hospice care**, while **controlling symptoms** and **providing emotional support**.
Opioid Analgesics	**Medications** such as **morphine** and **fentanyl**, considered as **essential** by **WHO** for **pain relief in palliative care.**
Noncommunicable Diseases (NCDs)	Leading causes of death worldwide; **Chronic illnesses** such as **cancer**, **cardiovascular diseases**, **diabetes**, and **chronic respiratory conditions**.
Quality of Life	The primary **aim** of **palliative care**: A measure of **overall well-being**, encompassing **physical**, **psychological**, and **social health**.
Universal Health Coverage	An **objective** by **WHO** and the **UN** to ensure that all individuals and communities receive the **health services** they **need** without going through **financial hardship**.
Pain Management	The use of **medications**, **therapies**, and **psychological support** to **reduce physical suffering** in patients.
Advance Care Planning (ACP)	Advance care planning involves making a **plan** for **future health** and **personal care** should a person lose their **decision-making capacity**.
Millennium Development Goals	**Eight** international development goals for the year **2015** created following the **Millennium Summit**, following the **adoption** of the **United Nations Millennium Declaration**

Euthanasia	
Term	***Definition***
Euthanasia	An **act** performed by a **physician** to **end the life** of a **terminally ill patient**, at the patient's **explicit request**.
Assisted Suicide/Dying	An act of **intentionally assisting** someone in **ending their life**, most of the time by providing means of **medication**. However, the **final action** is done by the **patients** themselves, **not the doctor**.
Active Euthanasia	A form of **euthanasia** where the **physician actively intervenes**, for example, by **administering a lethal injection**.
Passive Euthanasia	A form of **euthanasia** where the **physician withdraws life-sustaining treatment**, such as **turning off a respirator**.
Voluntary Euthanasia	**Euthanasia** carried out with the **full consent explicitly mentioned by the patient**; typically used in cases of **terminal illness** or **severe suffering**.
Involuntary Euthanasia	**Euthanasia** performed **against** the patient's **wishes**, often considered **illegal, morally and ethically unacceptable; also considered a violation to human rights and dignity**.
Non-voluntary Euthanasia	**Euthanasia** that is performed when the **patient** is **unable** to give **consent**, which is promptly in s**erious cases of unconsciousness or incapacitation.**
Sanctity of Life	A belief that **human life** is **sacred** and **deserving** of **unconditional protection** and **respect**. This is usually used as an argument against the use of euthanasia, claiming that life, as a sacred gift, **should not be deliberately terminated.**
Right to Die	The **concept** where individuals should have the **right to choose the timing and manner of their death**, especially in cases of **terminal illness and intense suffering.**
Advance Directive of Living Will	A **document** issued **legally** to patients to **specify their medical preferences**, which also includes **end-of-life decisions**, if there are cases where the patients will no longer be able to communicate their decision later.

Millennium Development Goals (MDGs)

- **MDG 1** : Eradicate extreme **poverty** and **hunger**
- **MDG 2** : Achieve **universal primary education**
- **MDG 3** : Promote **gender equality** and **empower women**
- **MDG 4** : Reduce **child mortality**
- **MDG 5** : Improve **maternal health**
- **MDG 6** : Combat **HIV/AIDS**, **malaria** and other **diseases**
- **MDG 7** : Ensure **environmental sustainability**
- **MDG 8** : Develop a **global partnership** for **development**

Sustainable Development Goals (SDGs)

1. No Poverty
2. Zero Hunger
3. **Good Health and Well-Being**
    a. **Ensure healthy lives and promote well-being for all at all ages**

        3.1 : By 2030, reduce global maternal mortality ratio to less than 70 per 100,000 live births

        3.2 : By 2030, end preventable deaths of newborns and children under 5 years of age, with all countries aiming to reduce neonatal mortality to at least as low as 12 per 1,000 live births and under-5 mortality to at least as low as 25 per 1,000 live births

        3.3 : By 2030, end the epidemics of AIDS, tuberculosis, malaria and neglected tropical diseases and combat hepatitis, water-borne diseases and other communicable diseases

        3.4 : By 2030, reduce by one third premature mortality from non-communicable diseases through prevention and treatment and promote mental health and well being

        3.5 : Strengthen the prevention and treatment of substance abuse, including narcotic drug abuse and harmful use of alcohol

        3.6 : By 2020, halve the number of global deaths and injuries from road traffic accidents

        3.7 : By 2030, ensure universal access to sexual and reproductive health-care services, including for family planning, information and education, and the integration of reproductive health into national strategies and programmes

        3.8 : Achieve universal health coverage, including financial risk protection, access to quality essential health-care services and access to safe, effective, quality, affordable essential medicines and vaccines for all

        3.9 : By 2030, substantially reduce the number of deaths and illness from hazardous chemicals and air, water and soil pollution and contamination

        3.a : Strengthen the implementation of the World Health Organization Framework Convention on Tobacco Control in all countries, as appropriate

        3.b : Support the research and development of vaccines and medicines for the communicable and non-communicable diseases that primarily affect developing countries, provide access to affordable essential medicines and vaccines, in accordance with the Doha Declaration on the TRIPS Agreement and Public Health, which affirms the right of developing countries to use to the full provisions in the Agreement on Trade-Related Aspects of Intellectual Property Rights regarding flexibilities to protect public health, and, in particular, provide access to medicines for all.

        3.c : Substantially increase health financing and the recruitment, development, training and retention of the health workforce in developing countries, especially in the least developed countries and small island developing States.

        3.d : Strengthen the capacity of all countries, in particular developing countries, for early warning, risk reduction, and management of national and global health risks.
4. Quality Education
5. Gender Equality
6. Clean Water and Sanitation

7. Affordable and Clean Energy
8. Decent Work and Economic Growth
9. Industry, Innovation and Infrastructure
10. **Reduced Inequalities**
    a. **Reduce inequality within and among countries**

        10.1 : By 2030, progressively achieve and sustain income growth of the bottom 40 per cent of the population at a rate higher than the national average

        10.2 : By 2030, empower and promote the social, economic, and political inclusion of all, irrespective of age, sex, disability, race, ethnicity, origin, religion or economic or other status

        10.3 : Ensure equal opportunity and reduce inequalities of outcome, including by eliminating discriminatory laws, policies, and practices promoting appropriate legislation, policies, and action in this regard

        10.4 : Adopt policies, especially fiscal, wage and social protection policies, and progressively achieve greater equality

        10.5 : Improve the regulation and monitoring of global financial markets and institutions and strengthen the implementation of such regulations

        10.6 : Ensure enhanced representation and voice for developing countries in decision-making in global international economic and financial institutions in order to deliver more effective, credible, accountable and legitimate institutions

        10.7 : Facilitate orderly, safe, regular and responsible migration and mobility of people, including through the implementation of planned and well managed migration policies.

        10.a : Implement the principle of special and differential treatment for developing countries, in particular least developed countries, in accordance with World Trade Organization agreements

        10.b : Encourage official development assistance and financial flows, including foreign direct investment, to States where the need is greatest, in particular least developed countries, African countries, small island developing States and landlocked developing countries, in accordance with their national plans and programmes.

        10.c : By 2030, reduce to less than 4 per cent the transaction costs of migrant remittances and eliminate remittance corridors with costs higher than 5 per cent
11. **Sustainable Cities and Communities**
    a. **Make cities and human settlements inclusive, safe, resilient and sustainable**

        11.1 : By 2030, ensure access for all to adequate, safe and affordable housing and basic services and upgrade slums
12. **Responsible Consumption and Production**
13. Climate Action
14. Life Below Water
15. Life on Land
16. **Peace, Justice, and Strong Institutions**
17. **Partnerships for the Goals**

Palliative Instead of Curative Medication

Honorable Chairs and Fellow Delegates, the debate of ethical and practical implementation of palliative care instead of curative medicine is one that is in urgent need of discussion. Belgium supports the integration of palliative care into general medicine which is shown by the large proportion of general hospitals containing a palliative care unit. The strengthening of systems that allow for the protection of patients and medical professionals is crucial in this debate to remain ethical. Furthermore, Belgium urges all nations to better collaborate and create standardized expectations and regulations such as age requirements, and informed consent to ensure ethics remain front of mind. At the same time, Belgium acknowledges the economic factors that limit treatment options which is why correctly allocating an adequate budget is crucial in implementing the care that is a fundamental human right to a quality life. Finally, in order to implement palliative care, regulations must be revised to liberalize the use of opioid medications for palliative care purposes and systems must be developed to find alternatives in the case that one medication is unavailable. This delegate yields the floor back to the chairs.

Euthanasia Legalization and Regulation

Honorable Chairs and Fellow Delegates, as the second nation in the world to legalize euthanasia, Belgium remains a crucial voice with more than 20 years of experience. Belgium remains supportive of ethical and compassionate euthanasia to alleviate suffering from those who can no longer achieve recovery. Belgium believes euthanasia should remain a choice and should be permitted for both physical and mental suffering. Belgium experienced roughly 20-30 cases per month after the initial implementation, due to the delayed palliative care. Belgium urges all nations to create stricter regulations for euthanasia and ensure the involved parties are well informed while maintaining early intervention and relying on patient autonomy to decide the best course of action. Belgium calls for comprehensive criteria and procedures around euthanasia, thus, the act of euthanasia must be done in a regulated setting and in the company of at least 2 medical professionals to minimize opportunity for malpractice. Furthermore, Belgium acknowledges the ethical debates and question of possible recovery especially for mentally ill patients however the key point of this debate is patient choice, if by all accounts a patient is capable of making and eligible to make the choice to die with dignity, who is to say that is not permissible? This delegate yields the floor back to the chairs.

(motion for 90 secs)

Position Paper

Position Paper : Belgium - WHO

Opening Speech

Honorable Chairs and Fellow Delegates, as the second nation in the world to legalize euthanasia, the delegation of Belgium believes that the integration of palliative care in place of aggressive and inappropriate curative care is necessary in order to allow patients to die with dignity and choose to prioritize their quality of life. This delegation acknowledges the dangers of euthanasia if done with undue process, despite this Belgium has had only 1 significant legal issue in 2012 indicating that euthanasia is used as intended and rarely abused. Belgium remains one of the only countries where euthanasia for mental suffering is accepted, with infrastructure and policies considering the possible implications on euthanasia and the health industry. Belgium urges all delegations to look at Belgium as an ideal situation where economic, social, and cultural factors are considered and where individual patients are the first and most important point of contact. With mass cooperation and standardization, globally, palliative care and euthanasia can be accepted and integrated with reasonable guidelines and limits to ensure autonomy and dignity at the end of life.

Resolution Ideas

1. **Urges** the development of global healthcare funding (especially for those under 18)
    a. **Encourages** the coverage of most if not all palliative care and euthanasia
2. **Calls for** the use of stricter, comprehensive requirements in the context of euthanasia
    a. **Demands** non-negotiable age limits
    b. **Demands** extensive screening procedures to assess individuals (especially for those experiencing mental suffering)

- Restrictions for euthanasia
    - Physical illness
        - No age limit – decision must be made by parents until the age of 15
        - At the age of 15 individuals can apply through court
        - Waiting period of 3 months for non-terminal cases for minors
        - Waiting period of 1 month for non-terminal cases for anyone over the age of 40
    - Mental illness
        - 10+ – decision must be made by parents until the age of 16
        - At the age of 16 individuals are able to apply through court
        - Waiting period of at least 6 months for minors
        - Waiting period of 1 month for anyone over the age of 40
- Euthanasia must only be requested after several attempts of palliative care are made
- Euthanasia must be requested multiple times with certainty
    - All requests must be fully voluntary and not a result of external pressures
- Alternatives to active euthanasia are equally encouraged, such as:
    - Deep sedation (as implemented by France)
    - Passive euthanasia
    - Assisted suicide
    - Physician assisted suicide
        - Trained non medical professionals assisted suicide
    - Voluntary assisted suicide
- Several psychological exams must be completed determining the sound mind and voluntary desire for euthanasia
- Religion is respected and specialized psychotherapy may be offered to come to terms with euthanasia as an act of compassion
- Resource limits must be set to ensure terminally ill patients with no hope of recovery are not draining resources – that must be prioritized for patients that will recover
- Non voluntary suicide is expressly banned and considered a form of homocide
- All requesting patients must complete a certain number of psychological counselling sessions to ensure the desire is persistent and well considered

    - All counselling must be unbiased and neutral on the act of euthanasia, simply encouraging reflection and assisting in alternative options
- All requests for euthanasia must be submitted with evidence of economic status to ensure external factors such as the costs of life sustaining care are considered and not influencing the ultimate choice of euthanasia

Bloc Ideas

3. **Demands** the development of tracking systems
    a. **Uses** technology to track physical symptoms, pain, and treatments
    b. **Uses** technology to track social, economic, and cultural factors (that may influence the patients care choices

        (**Belgium,**

4. **Strongly encourages** the liberalization of policies regulating opioid use for palliative care purposes

    (**Colombia**, **Belgium,**

5. **Encourages** the development of alternative care options (such as psychedelic-assisted therapy) for individuals experiencing mental suffering
6. **Urges** the development of global healthcare funding (especially for those under 18)
    a. **Encourages** the coverage of most if not all palliative care and euthanasia

        (**Belgium,**

7. **Calls for** the use of stricter, comprehensive requirements in the context of euthanasia
    a. **Demands** non-negotiable age limits
    b. **Demands** extensive screening procedures to assess individuals (especially for those experiencing mental suffering)

        (**Belgium,**

- Use of verified third parties to administer medicines
    - Training available for individuals wanting to help administer medications (license is available until death/end of prescription whichever occurs first)

        (**Belgium,**

- Use of verified messenger services to ship necessary medications especially to rural areas

    (**Belgium,**

- Restrictions for euthanasia
    - Physical illness
        - No age limit – decision must be made by parents until the age of 15
        - At the age of 15 individuals can apply through court
        - Waiting period of 3 months for non-terminal cases for minors
        - Waiting period of 1 month for non-terminal cases for anyone over the age of 40

            (**Belgium,**

- Mental illness
    - 10+ – decision must be made by parents until the age of 16
    - At the age of 16 individuals are able to apply through court
    - Waiting period of at least 6 months for minors
    - Waiting period of 1 month for anyone over the age of 40

(**Belgium,**

- Curative care should be implemented until :
    - The patient requests for it to stop
    - The patient experiences constant pain or side-effects for over 3 months with no signs of improvement
    - The hope for recovery is proven to be non-existent

(**Belgium,**

For Speeches

This resolution utilizes given advantages to overcome challenges while maintaining human rights and safety. ***Clause 10 Supports*** the use of verified messenger services to ship necessary medications especially to rural areas and LMICs. Verified courier services can be implemented to help international mobilization of essential medications and bridge an access gap by providing less developed systems with funding, resources, and training all of which may be sourced from a combination of government subsidies, taxations, and funds collected by international organizations, such as: the WHO, International Monetary Fund, the World Bank, and private investors to fund palliative care and preventative health programs. Provided resources may include those mentioned in Clause 8 such as; essential medications, and emergency kits. Furthermore, this resolution provides comprehensive and diverse perspectives that implements creative solutions that consider all involved parties and specific circumstances.

Against Speeches

Main Sub Speeches

3,4,9,5

03) Urges for the creation of policies governing the administration of euthanasia, such that it cannot take place unless all policies are met, thereby preventing any slippery slopes or worsening of social issues regarding euthanasia:

- **The patient is suffering with unbearable pain, and facing challenges in their daily life,**
- Every possible palliative treatment and care to ease the patient's physical pain and suffering has been provided and no alternatives are available or waiting lists with treatments,
- **The patient has expressed a clear, repeated, and voluntary desire to have his or her life shortened,**
- The patient has previously filled out a will for either or both passive and active euthanasia, thus limiting them to receiving that type of euthanasia,
- The patient's decision must be made when they are fully conscious and after they are informed of all possible options offered not only in their hospital, but in the entire medical community, and the patient must be consulted alone before consulting family members,
- The patient must not be on the waiting list for any other treatment and the list of every possible palliative treatment to try on the patient must be exhausted and unsatisfactory to the patient,
- **Ensuring poverty wasn't a driving force,**
    - **Requires the submission of economic status documents to assess whether the choice is being implemented without the influence of financial hardship**
- The patient must be suffering from unbearable pain, and facing challenges in their daily life,
- The patient must have an assessment conducted for mental illness, the presence of which would direct them for further psychological assessment,
    - To assess the capability of the individuals to make their own decisions
- The patient must consent to the euthanasia again prior to administration of active euthanasia,
- **The patient meets guidelines for age restrictions and waiting periods, including:**
    - **Guidelines for physical illness**
        - No age limit – decision must be made by parents until the age of 15,
        - At the age of 15 individuals can apply through court
        - Waiting period of at least 3 months for non-terminal cases for minors
        - Waiting period of at least 2 month for non-terminal cases or for anyone over the age of 40
    - **Guidelines for mental illness**
        - 10+ – decision must be made by parents until the age of 16
        - At the age of 16 individuals are able to apply through court
        - Waiting period of at least 6 months for minors
        - Waiting period of 1 month for anyone over the age of 40
    - **The patient must not be treated with euthanasia as a "cry for help",**
    - **The patient is not vulnerable,**
    - **The patient is not making the decision due to feeling like a burden on their family,**
    - **The patient does not request euthanasia due to a passing phase of the disease,**
    - **The patient must acknowledge alternatives to euthanasia before allowing conversations surrounding euthanasia**
        - **Deep sedation, as implemented by France**
        - **Passive euthanasia**
        - **Assisted suicide**
        - **Physician assisted suicide**
        - **Voluntary assisted suicide**

04) Suggests the regulation and training of the physician code of conduct through various means such as but not limited to:

- Monitoring of physician behavior towards patients, creating an easy way to report physicians in person or through the IAPC-HP,
- **Creating severe legal consequences for any euthanasia encouragement**
- Training physicians in late medical school how to be compassionate towards patients and create transparency and honesty,
- Encouraging physicians not to be the first to propose or mention euthanasia as an option, meaning the patient must have had to think through their decision and openly express their feelings to end their life to their doctors,
- **Employment of doctors should take aptitude and ethical responsibility into account,**
    - **Background checks have been performed and continue to be performed every 2 years showing no signs of necessary caution**

05) Suggests the clarification of euthanasia before execution on administration day through:

- **Encouraging individuals to customize their site and environment for death,**
- Sign a waiver with terms and conditions stating the consequences of euthanasia that include:
    - Immediate death,
    - Possible stress on close relations,
    - Finances,
- **Space and time provided between the registration in the beginning and the final administration,**
- Final clarification before delivery,

09) Permits all types of euthanasia with all being last resorts and following other aforementioned guidelines, including:

- Active euthanasia, with alternatives to it being encouraged,
- Passive euthanasia; permitted only if the patient requested for DNR prior to the incident and official form is recorded in a digital platform, with types being:
    - **withdrawing life supporting machines**
    - **refraining from resuscitation**
    - **Physician assisted suicide**
    - **Assisted suicide**

Belgium believes that the only way for euthanasia to be ethically implemented is through strict criteria of which is detailed in clause 3 through the guidelines around both mental and psychical suffering. The resolution states :

- **The patient is suffering with unbearable pain, and facing challenges in their daily life,**
- **The patient has expressed a clear, repeated, and voluntary desire to have his or her life shortened,**

- **We believe that after all possible methods of treatment have been exhausted, euthanasia is the most appropriate option. Passive euthanasia causes prolonged suffering and heightens pain.**

To force individuals to endure unbearable pain is far more unethical than ending suffering. To be alive is to actively participate in society. Patients being forced to undergo aggressive and unnecessary treatments are lacking opportunity to participate in the world around them. How valuable is the sound of a heart monitor and should it override personal choice? Studies show patients are more likely to take it upon themselves to end their life when deprived of a legal and moral path to euthanasia. Furthermore, the act of suicide is likely to cause trauma for people who find the body. Finally, the consideration of treatment resistant conditions are lacking and fail to address that palliative care is not always effective and leaves patients with treatment resistant conditions alone.

Other Speeches

Economic Benefits & Drawbacks of Palliative Care

Belgium acknowledges that the impact of palliative care on the economy is generally positive with overall lower costs for palliative care than curative treatments. The reasons for the lower costs include: reduced in-patient stays, minimal diagnostics, and fewer hospital admissions. Belgium believes the ethical implementation of palliative care at the appropriate stage of treatment is a necessary transition in order to redirect the medical economy and ensure costs are being appropriately allocated with focus on equitable and accessible care. Though the costs of palliative care are lower than those of curative treatments, they are not affordable for everyone. Thus, Belgium strongly encourages all nations to develop national healthcare systems that cover palliative care as well as curative treatments with emphasis on vulnerable populations such as immigrants and elderly patients. Through the development of diverse and adapted national healthcare systems, we can work towards ensuring all people have access to affordable, quality care.

Motion for 60 seconds speaking time
52 secs || 50 secs || 51 secs || 49 secs

The Regulation of Palliative Care

Belgium believes that international standards for palliative care must be described and implemented to ensure ethical decisions are being made in the best-interest of the patient while ensuring progress in the integration of palliative care into general medicine is being made. Crucially, palliative care must be seen as an addition to curative care – based on informed patient decisions and physician recommendations. It must never be a replacement. Furthermore, though palliative care remains controversial as it does not aim to cure the present condition – which some may perceive as failing to prevent death, palliative care aims to better the patient's quality of life and has even shown to extend life past prognosis. Belgium's palliative care is being actively integrated into general practice – as suggested by the WHO, this means the need for regulation and standards is urgently demanded as broader groups become involved in the process. The need for training in palliative care is prominent with many nations lacking widespread education, exacerbating the existing challenge of equitable access in rural areas. Thus, with the implementation of international standards, nations can further develop community based care and engage in deinstitutionalization to finally bridge the access gap between urban and rural communities.

Motion for 90 seconds speaking time
1 min 13 secs || 1 min 12 secs || 1 min 8 secs || 1 min 6 secs

The Access to Both Palliative and Curative Care

Belgium strongly supports the integration of palliative care into general medicine but remains firm in the fact that palliative care is not a replacement for curative treatments. Belgium urges all nations to consider the need for equal and proportionate access to both palliative and curative care. Furthermore, the current funding is severely lacking which has been emphasized throughout the pandemic, which is why the use of international collaboration to support LMICs is crucial. Through transparent communication with patients, medical professionals can help determine when palliative care is a more appropriate option. For example, Belgium believes that patients with poor prognosis and specified clinical indicators should be treated with palliative care as the chance of recovery is slight if not none, in which case comfort and quality of life should be prioritized. With international collaboration, nations must define and implement – through legislation – the specified occurrences where palliative care is a more appropriate option in order to implement a balance of palliative and curative treatments.

Motion for 60 seconds speaking time
53 secs || 50 secs || 49 secs || 51 secs

The Liberalization of Opioid Medications for Palliative Care Purposes

The use of opioid medications for palliative care purposes remains encouraged by the WHO, with efforts to liberalize the use of opioid medications to ensure care is as accessible as possible. Belgium acknowledges this need for liberalization however urges caution in doing so due to the various opioid crises across the globe. Belgium believes that policies and systems must be developed to allow verified non-medical personnel (third parties) such as; trusted family, hospice volunteers, and community health workers – to collect and administer opioid medications after completing an international standardized course. Furthermore, the development of secure packaging and courier services will minimize the access gap in rural areas or in less developed countries. Belgium urges all higher income countries to consider regional or international collaborations to provide legitimate opioid medications to lower income countries who may be lacking in resources and professionals. Through the development of new technologies, systems, organizations, and educational courses, we can safely liberalize opioid medications without encouraging abuse or addiction.

Motion for 60 seconds speaking time
59 secs || 1 min 2 secs || 56 secs || 57 secs

Balancing Palliative and Curative Treatments

The delegation of Belgium has an actively developing palliative care systems with various infrastructure like mobile care teams to aid patients in receiving the most preferred, and appropriate care. Palliative care should not replace nor be balanced with curative care. Patients should be given the opportunity to choose care pathways and choose the sites of care. Crucially, defining criteria around what qualifies as appropriate curative care and questioning when care is aggressive and has little purpose. Belgium encourages the consideration of elderly individuals as curative treatments have little value for people who are dying (naturally or due to illness). Primarily, Belgium urges the careful evaluation of patient needs to determine the best path of care. Furthermore, the development of palliative care teams to help individuals plan their care with personal preferences in mind. Through patience and understanding, the WHO can guide all nations in developing better palliative care systems and international guidelines.

Ethical Considerations of Euthanasia

The act of euthanasia in Belgium has long been legalized, as such; ethical considerations had to be made. For example in the Belgium Act on Euthanasia, they state that physicians cannot be compelled to perform euthanasia and other persons cannot be forced to assist in it. This is a basic but necessary consideration; however, more considerations must be made and implemented through legislation. Belgium believes that reform is necessary to show an example of an effective system for euthanasia. Belgium encourages stricter conditions for euthanasia, most specifically age restrictions as minors should not be eligible for euthanasia, with the exception of severe terminal cases where any curative treatment will cause more harm, though this must be approved by a governed body. Furthermore, considerations must be made for families, thorough processes must be followed to ensure relatives are consulted. On the contrary, requests for family to not be informed until after may be made and accepted under strict circumstances such as estrangement. Finally, religious considerations must be made as many religions struggle to accept euthanasia as an act of compassion as it intentionally ends one's life which goes against the various religious texts and guidelines. Through the understanding of religious beliefs and priorities, nations can collaborate to define necessary conditions and limits of euthanasia.

Motion for 90 seconds speaking time
1 min 10 secs || 1 min 9 secs || 1 min 10 secs || 1 min 10 secs

Cultural Influence on Euthanasia Legislation

The cultural influences on euthanasia vary nation to nation, however religion remains the most prominent factor. Since Belgium has a relatively high proportion of agnostics or atheists, Belgium encourages collaboration with the Special Rapporteur on freedom of religion or belief to inform legislation surrounding euthanasia to ensure policies are inclusive and generally accepted. Furthermore, cultural traditions and dynamics may influence the weight of familial

opinions or beliefs in the afterlife. These factors should not be neglected, but instead incorporated into legislation to show acknowledgement and to include all nations regardless of cultural influence. Critically, the mistrust in healthcare in certain communities and regions must be accounted for and rectified through patience and understanding, policies must meet people halfway. However, certain lines must be drawn to indicate when external pressures are interfering with the best interests of the patient, thus, the development of new national and international policies, and governing bodies can help repair the relationship between culture and modern bio-medicine.

Motion for 60 seconds speaking time
57 secs || 54 secs || 49 secs || 49 secs

The Regulation of Euthanasia

Defining the regulations and conditions for euthanasia is a complex task, primarily because diverse opinions and cultural influences on euthanasia have prevented the development of international standards. Belgium believes that for euthanasia to remain a compassionate act of care, strict regulations are essential in ensuring ethical standards are being met and human rights are being protected. Therefore these conditions for illnesses must be mandated:

- First, several medical consultations must be completed to determine the possibility of recovery
- Second, the use of a research team to assist in discovering new treatments is encouraged
- Third, patients must be experiencing unbearable suffering that cannot be alleviated with essential medications
- Fourth, patients must have a medically futile condition
- Lastly, patients must have evidence of symptoms that have been experienced for a specified period of time (depending on the condition).

These criteria alongside more comprehensive limitations must be implemented to protect patient autonomy while allowing for ethical practices of euthanasia. Through international collaboration, national education systems, and discussion, we can make euthanasia a compassionate and accessible act while considering the ethical and cultural factors that must be actively accounted for.

Motion for 90 seconds speaking time
1 min 6 secs || 1 min 4 secs

Active Euthanasia versus Passive Euthanasia

Belgium believes both active and passive euthanasia should be permitted especially as many severely ill patients are incapacitated and incapable of administering the lethal medication themselves. Belgium also highlights that active euthanasia is developed to ensure pain is minimal and the fact is most often active euthanasia is less painful and passive euthanasia and limits suffering to minutes instead of months or even years.

Euthanasia for the Purpose of Ending Mental Suffering

Euthanasia for mental suffering is a controversial topic that very few countries have addressed. Belgium believes that euthanasia is a compassionate act and should not be limited to patients experiencing physical suffering. Belgium's statistics show that only 8% of euthanasia cases stem from the sole reason of mental illness. Many fear that expanding the law to encompass mental suffering as an eligible path to euthanasia is a slippery slope, with conflicts regarding will to live and possibility of recovery. However, this is unlikely as strict conditions must be met to qualify as a patient with incurable mental illness. A key challenge Belgium highlights is that neurobiology and psychology are both relatively new fields. Opportunities for innovation are greater; however, this may complicate the decision to end one's life as there is no predicting the rate of development for new alternative treatments or cures. Therefore, Belgium urges all nations to consider how to safely implement pathways for euthanasia due to mental suffering. Belgium strongly encourages nations to ensure the decisions to terminate a patient's life are made with no external pressures, and with fully informed consent, as well as ensuring patients meet all comprehensive criteria.

Motion for 90 seconds speaking time
1 min 12 secs

Child Euthanasia

The act of euthanasia for minors is a topic on which Belgium's stance differs from most. Belgium believes with parental consent and the meeting of other essential criteria, child euthanasia may be permitted. For terminally ill children, there should be no reason to extend their suffering nor the parents grief if there is no hope of recovery. Alongside this decision, support must be given for children (if appropriate) and for grieving families, support for acting physicians or involved parties may be offered upon request as the act of ending a child's life is extremely inappropriate in all other circumstances. Belgium acknowledges that there is a possibility of new treatment development however the draining of resources for decades is inappropriate and unethical considering the existing challenge of access for many individuals across the world. Belgium urges all nations to consider severe terminal cases in children for euthanasia provided strict conditions are met and parent consent is given. Lastly, Belgium also highlights that this is simply a last resort and should not occur until all treatment options and palliative care is implemented.

Bullet Points

The Regulation of Palliative Care

- The **development** of **international standards** to ensure **ethical decisions** are being made
    - Decisions made in the **best interest** of the **patient**
    - Ensuring **integration** is **developing** reasonably
- Palliative care must be an **addition not replacement** for curative care
    - Palliative care should be **implemented** based on the **informed patient decisions** and **physician recommendations**
- Palliative care has even shown to **extend life past prognosis**
- Palliative care **training** is **lacking worldwide** – this must be rectified
- **Challenges** include **equitable access** in **rural areas**

Economic Benefits & Drawbacks of Palliative Care

- Palliative care has a generally positive economic impact
    - Overall lower costs compared to curative treatments
    - Due to shorter in-patient stays, minimal diagnostics, and fewer hospital admissions
- Ethical implementation is essential
    - Should be introduced at the appropriate stage of treatment
    - Helps redirect the medical economy toward equitable and accessible care
- Affordability remains a concern
    - Although cheaper overall, not affordable for everyone
    - Particularly affects vulnerable populations such as immigrants and the elderly
- Call for national healthcare systems
    - Should cover both palliative and curative care

    - Must focus on equity, accessibility, and quality
    - Encourage diverse and adapted healthcare systems across nations
- Goal: universal access to affordable, quality care
    - Ensure resources are allocated efficiently
    - Promote ethical and inclusive healthcare development

The Access to Both Palliative and Curative Care

- Belgium supports integrating palliative care into general medicine
    - Emphasises that palliative care must complement, not replace, curative treatments
    - Seeks balanced access to both forms of care
- Equal and proportionate access is essential
    - All nations should ensure fair availability of both palliative and curative services
    - Highlights the need to reduce inequality in healthcare provision
- Funding for palliative care is insufficient
    - Severely lacking, especially revealed during the COVID-19 pandemic
    - Calls for greater investment in palliative services
- Transparent communication with patients is crucial
    - Medical professionals should guide patients in choosing the most appropriate treatment
    - Decisions should be based on clinical indicators and patient prognosis
- Prioritising comfort and quality of life
    - For patients with poor prognosis or minimal recovery chance, palliative care should be

prioritised

◦ Focus on dignity, comfort, and ethical treatment

- Need for international collaboration and legislation

◦ Nations should define and legislate criteria for when palliative care is the more suitable option

◦ Aim to balance palliative and curative treatments globally

The Liberalization of Opioid Medications for Palliative Care Purposes

- WHO encourages the use of opioid medications for palliative care

◦ Aims to liberalize access to ensure treatment is widely available

◦ Supports accessible and effective pain management globally

- Belgium acknowledges the need for liberalization but urges caution

◦ Recognises the risk of opioid misuse and addiction

◦ Notes ongoing opioid crises in several countries as a warning

- Call for regulated systems and verified third-party involvement

◦ Policies should allow trained non-medical personnel (e.g. family members, hospice volunteers, community health workers)

◦ Must complete an international standardized course before handling or administering opioids

- Improving accessibility through secure distribution

◦ Develop secure packaging and courier systems

    - Helps bridge access gaps in rural areas and developing countries
- International collaboration for equity in medication access
    - High-income countries should collaborate to supply legitimate opioid medications to low-income nations
    - Focus on safe distribution and ethical access
- Innovation and education as safeguards
    - Promote new technologies, systems, and organizations to ensure safety
    - Develop educational courses to prevent abuse and addiction
    - Goal: safe liberalization without increasing risk of opioid dependency

Ethical Considerations of Euthanasia

- Euthanasia is legal in Belgium
    - Governed by the Belgian Act on Euthanasia
    - States that physicians cannot be compelled to perform euthanasia
    - No person can be forced to assist in the process
- Need for ongoing ethical reform
    - Belgium supports further legislative development to ensure ethical integrity
    - Seeks to set an example of a transparent and effective euthanasia system
- Stricter conditions and eligibility criteria
    - Age restrictions should be reinforced
    - Minors should not be eligible, except in severe terminal cases
    - Such cases must be reviewed and approved by a governing body
- Family involvement and consent

    - Families should be consulted in all euthanasia processes
    - Exceptions may apply if the patient requests non-disclosure due to estrangement or similar justified reasons
    - These exceptions must be strictly regulated and approved
- Religious and cultural considerations
    - Recognises that many religions reject euthanasia as it conflicts with spiritual beliefs about life and death
    - Encourages interfaith dialogue and international collaboration to define ethical boundaries and conditions
- Goal: balanced, ethical, and respectful euthanasia legislation
    - Promote compassionate end-of-life care while respecting moral, familial, and religious values

Cultural Influence on Euthanasia Legislation

- Cultural and religious influences on euthanasia
    - Vary from nation to nation, with religion being the most significant factor
    - Belgium recognises its high proportion of agnostics and atheists, shaping a more secular approach to euthanasia policy
- Collaboration with international bodies
    - Belgium supports working with the UN Special Rapporteur on Freedom of Religion or Belief
    - Goal: ensure legislation is inclusive, culturally sensitive, and broadly accepted
- Incorporating cultural traditions into policy
    - Cultural values, family dynamics, and beliefs in the afterlife influence end-of-life decisions
    - These factors should be acknowledged and reflected in national legislation

    - Ensures representation of all nations regardless of cultural or religious context
- Addressing mistrust in healthcare systems
    - Some communities show mistrust toward medical authorities
    - This must be addressed through patience, transparency, and understanding
    - Policies should bridge the gap between traditional beliefs and modern medicine
- Balancing cultural respect with patient autonomy
    - External pressures (family, community, or religious) must not override patient welfare
    - New national and international policies should establish clear ethical boundaries
    - Encourage governing bodies to oversee and protect patient-centred decisions
- Goal: harmony between culture and bio-medicine
    - Foster mutual respect between cultural traditions and modern ethical healthcare practices

Motions

- **Motion to open debate on the question of implementation and usage of palliative care instead of curative medication**
- **Motion to open debate on the question of the regulation and legalization of euthanasia**

- **Motion to extend speaking time to 90 seconds**
- **Motion for __ minutes unmoderated caucus**
- **Motion for __ minutes moderated caucus on the topic of __, with __ seconds per speaker**
- **Motion for consultation of the whole for __ minutes on the topic of __**
- **Motion to open debate on resolution __**
- **Motion to adjourn session**
- **Motion to close debate on resolution __**

Topics for Mod or Consultation

Palliative VS. Curative Medicine

- The economic benefits and drawbacks of palliative care (**60 secs**)
- The regulation of palliative care (criteria, limitations, laws) (**90 secs**)
- The access to palliative and curative care (**60 secs**)
- The liberalization of opioid medication for palliative care purposes (**60 secs**)

Euthanasia

- Ethical considerations of euthanasia (**90 secs**)
- Cultural influence on euthanasia (**60 secs**)
- The regulation of euthanasia (criteria, limitations, laws) (**secs**)
- Euthanasia for the purpose of ending mental suffering (**secs**)
- Extension of the law to encompass minors (**secs**)

POIs

This delegate would like to ask through the chairs,

- How does the delegate plan on ___?
- How does the delegate aim to __?
- Why does the delegate believe __ is necessary?
- What funding does the delegate intend on receiving?
- How does the delegate believe __ will have a positive impact?

POCs

This delegate would like to ask through the chairs,

- What does the delegate mean by __?
- Can the delegate elaborate on the impact of __?

- **POI - point of information**
    - **This delegate would like to ask through the chairs** ___
    - Follow up (wait for it to be granted, 1x only)
- **POC - point of clarification**
    - **This delegate would like to ask through the chairs** ___

Topics

Resolution POIs/POCs

- Does the <u>resolution</u> **allow** room for **regional agreements** and **national laws** and **application**?
    - **(FRIENDLY)**

- Can the <u>resolution</u> be **adapted** to suit **international circumstances**?
    - **(FRIENDLY) OR (UNFRIENDLY)**

- Does the resolution account for; **LICs**, **LMICs**, **MICs**, **UMICs**, **HICs**?
    - **(FRIENDLY) OR (UNFRIENDLY)**

- What is the **time frame** for this resolution?
  - **(UNFRIENDLY)**

- What is the **method** of **implementation** and **monitoring** for this resolution?
  - **(FRIENDLY)** **OR** **(UNFRIENDLY)**

- Does this resolution **account** for **national perspectives**?
  - Does this resolution **account** for **religious perspectives**?
    - **(FRIENDLY)** **OR** **(UNFRIENDLY)**
  - Does this resolution **account** for **political perspectives**?
    - **(FRIENDLY)** **OR** **(UNFRIENDLY)**
  - *(Are there any **clauses** that **violate** a **nation's policies** or **beliefs**?)*

- *(Are there any **violations** of the **UN Charter** or **international agreements**?)*

- *(What **parts** can be **strengthened**?)*
  - ***(How** can they be **strengthened**?)*

- *(Are the **supporters** of the **resolution aligned** with their **country's stance**?)*

- What are the **implications** on **national sovereignty**?
  - **(UNFRIENDLY)**

- How can **implications** on **national sovereignty** be **outweighed** by the **benefits** of the resolution?
  - **(FRIENDLY)**

- How can **implications** on **national sovereignty** be **outweighed** by the **benefits** of the resolution?

Palliative Care

Country	Notes
Argentina	● Economic crisis affected health care ● Rural regions struggle with access ● Advancing both palliative and curative care ● Expanded training for health care workers ● Early implementation of palliative care ● Human right ● Urban areas have reasonable access ○ Rural areas lack access ● Improved through aid from developed nations and regions
Australia	● 14% of patients who need palliative care receive ● Access to pain relief medication ● Public awareness – better accurate understanding
Belgium	—
Canada	● Human right ● Essential part of health care system ● Palliative and curative care work together ● 1970's initiation of palliative care ● Improving access to high quality palliative care ● Training of healthcare professionals
China	● Comfort must not be forgotten ● Expanding access to palliative care ● Rural areas struggle ● Increased international collaboration ● Community based care ● Live and die with dignity
Colombia	● Accessible palliative care ● First latin american country to implement palliative care ● Invest in accessible and affordable care ● Lack of education ● Lack of resources ● Institutional training for palliative care ● Equal opportunities

Palliative Care

Country	Notes
France	● Inaudible
Germany	● Managing symptoms ● Both curative and palliative care ● Developing countries lacks access ● HICs help LMICs with access ● Universal health coverage for palliative care ● Investment ● Tracking of care
Ghana	● No one deserves to die in pain ● Urgent need for palliative care ● Access is limited in rural areas ● Human right – not a luxury ● Help in training ● Standardized palliative care ● Healthcare coverage ● Training for medical professionals
India	● National programmes ● Government initiatives ● Integrate palliative care into general medicine ● Accessible care in rural areas ● Equitable access ● Human right ● International collaboration ● Train medical professionals ● Access to pain relief medication
Japan	● Pain relief ● Cultural background ● Encourages palliative care especially considering the declining birth rates ● Opioid addiction crises ● Home care ● Palliative care training ● Universal coverage ● Patient centered

Palliative Care

Country	Notes

	● Equity not equality
	● Government subsidies
	  ○ Funding from
	  ■ Taxes
	  ■ Global health funds
	  ■ Private investors
	● 40% of healthcare lost to inefficiencies

Kenya	● Alongside curative care ● Health care policies development ● Facilities in hospitals ● Increasing funding ● Supporting all involved parties ● International funding ● HICs aid in funding ● Curative treatments prioritized due to insufficient palliative care resources
Netherlands	● Everyone should practice palliative care ● Hospital facilities ● International training programmes ● Yearly reports to track progress
Norway	● Cure & care ● Die of unnecessary pain? ● Integration of palliative care into national health systems ● Accessible care ● Universal health care guarantees access to both curative and palliative care ● Treated ethically ● Increase funding ● Training professionals ● Raising awareness ● Compassionate care
Poland	● Not only curing, caring too ● Terminal or chronic illness suffering due to lack of developed systems ● Not giving up hope – choosing comfort when cure is no longer possible ● Integration into national health care systems

Palliative Care

Country	Notes
	● Access to pain relief medications
Portugal	● Curative replaced with palliative ● Challenges in home care ● Collaboration with UN and EU ● Palliative care alongside curative care

Russia	● Vital component of healthcare
	● Cautions against misuse and replacement of curative care by palliative care
	● Global guidelines to
	  ○ Protect patients
	  ○ Train professionals
	  ○ Provide accessible care

South Africa	● Compassionate care ● Integration of palliative care into all levels of general medicine ● Palliative care and curative care equal and should be balanced ● Affordable access to pain relief ● Raising awareness ● Equal access ● Palliative medicine scholarship programme ● Palliative services integrated into general medicine
Spain	● Medical and ethical failures ● Free access to palliative care ● Challenges with training and access ● Funding of palliative care and integration
Sweden	● Psychological and social support ● Relies on NGOs ● Accessibility is addressed through a formal suggestion by the king ● Patient dignity ● Ensuring better mental health ● Underdeveloped countries ● Therapy and psychological support ● Financial aid to LMICs ● Mobile support increased ○ Telemedicine

Palliative Care

Country	Notes
Switzerland	● Fundamental human right ● All levels of care ● Not only for terminal patients but for chronic illnesses as well ● Underfunding and limited coverage ● Expand access ● Training & research ● Collaboration with lower and middle income countries

Thailand	● Both palliative and curative care
	● Expanding and developing palliative care
	● Ongoing research
	● Challenges :
	  ○ Rural areas
	  ○ Lack of access
	  ○ Inadequate professional training
	● Calls for the help of HICs
	● Raise public awareness on palliative care

Uganda	● Continued suffering without pain relief in developing countries ● Integrates palliative care into general medicine ● Compassion and cure are equal ● Human right ● Medicines are unavailable → indicated a need for new medications to be developed

United Kingdom	● Comfort and compassion prioritized
	● Smooth stress free experience
	● Demand for palliative care is expected to rise
	● Curative medicine should remain the priority
	● Palliative care should be a support not replacement
	● International collaboration
	● Palliative care should be integrated in general medicine worldwide
	● Medical training and funding
	
	● Rural areas and lack of resources
	  ○ Funding and precise control
	  ○ Lack of trained professionals
	  ○ Appropriate use of palliative care (patient prioritization)

Palliative Care	
Country	**Notes**
United States of America	● Further implementation ● Balance between both curative and palliative care ● Hospice care – (NOT palliative care) ● Patient centered ● Early implementation ● Access to pain relief ● Crucial for all nations ● Equity is important ● Support for LMICs and vulnerable populations ● Expanding training programs in LMICs